

Section 5 Source Pack

Project: Destynnee dissertation, Section 5 Focus: Correctional rehabilitation models and rationale for an integrated treatment approach Purpose: Practical literature bank for drafting Note: I prioritized peer-reviewed empirical studies, meta-analyses, systematic reviews, and strong review articles. A few APA details should be verified before final submission, especially page ranges/issue numbers where I could not safely confirm them from available search results.

1) Evolution from punitive to rehabilitative correctional models

1. Lipsey, M. W., & Cullen, F. T. (2007). The effectiveness of correctional rehabilitation: A review of systematic reviews. *Annual Review of Law and Social Science*, 3, 297-320. <https://doi.org/10.1146/annurev.lawsocsci.3.081806.112833>

Plain-English summary: This is one of the clearest high-level reviews showing that rehabilitative interventions generally outperform punishment-only approaches when the outcome is recidivism reduction. Lipsey and Cullen synthesize decades of meta-analytic findings and argue that treatment effects are not huge, but they are consistent and meaningful. The paper is especially useful for showing that the old “nothing works” narrative was empirically overstated. How to use in Section 5: Use this as a foundational citation for the field’s shift away from pure punishment and toward structured rehabilitation.

2. Cullen, F. T., Jonson, C. L., & Nagin, D. S. (2011). Prisons do not reduce recidivism: The high cost of ignoring science. *The Prison Journal*, 91(3 Suppl.), 48S-65S. <https://doi.org/10.1177/0032885511415224>

Plain-English summary: This article argues that incarceration by itself is a weak crime-reduction tool and may even worsen outcomes under some conditions. The authors contrast deterrence-heavy assumptions with evidence that recidivism is better addressed through rehabilitative programming and risk-focused intervention. It is a strong bridge source when arguing that custody alone is not treatment. How to use in Section 5: Use this to justify why a treatment manual is needed instead of relying on confinement, sanctioning, or control alone.

3. Gendreau, P., Goggin, C., & Cullen, F. T. (1999). The effects of prison sentences on recidivism. User Report/Department of the Solicitor General Canada; findings later widely cited in peer-reviewed rehabilitation literature. APA details should be verified if cited directly.

Plain-English summary: This classic synthesis found little evidence that longer or harsher custodial sanctions reduce reoffending and some evidence that they may slightly increase it. Even though this source is older, it remains influential in the “punishment versus rehabilitation” debate because it directly tests deterrence assumptions. It helps demonstrate why correctional systems gradually moved toward treatment-oriented approaches. How to use in Section 5: Use this as historical empirical support that punitive intensity alone is not an evidence-based route to desistance.

4. Smith, P., Gendreau, P., & Swartz, K. (2009). Validating the principles of effective intervention: A systematic review of the contributions of meta-analysis in the field of corrections. *Victims & Offenders*, 4(2), 148-169.

<https://doi.org/10.1080/15564880802612581>

Plain-English summary: This review shows how the rehabilitation literature matured from broad “works/doesn’t work” questions to more precise evidence about which programs work, for whom, and under what conditions. It supports the evolution from generic correctional control toward targeted interventions built around empirically supported principles. The article also helps position correctional treatment as a science-driven field rather than an ideology-driven one. How to use in Section 5: Use this to transition from the historical shift toward rehabilitation into modern evidence-based frameworks.

2) Major rehabilitation frameworks including RNR and GLM

1. Andrews, D. A., Bonta, J., & Hoge, R. D. (1990). Classification for effective rehabilitation: Rediscovering psychology. *Criminal Justice and Behavior*, 17(1), 19-52. <https://doi.org/10.1177/0093854890017001004>

Plain-English summary: This is a cornerstone article for the Risk-Need-Responsivity (RNR) model. It argues that intervention works best when it matches service intensity to risk level, targets criminogenic needs, and is delivered in ways that fit the person’s abilities, motivation, and learning style. Even though it is older, it remains a key theoretical and empirical anchor for modern correctional treatment. How to use in Section 5: Use this to define the logic of RNR and ground later discussion of responsivity and individualized care.

2. Bonta, J., & Andrews, D. A. (2017). *The psychology of criminal conduct* (6th ed.). Routledge. Book source rather than journal article; pair with empirical RNR reviews below.

Plain-English summary: This book synthesizes the empirical literature supporting RNR and related cognitive-social learning approaches to criminal behavior. It is especially helpful for explaining how criminogenic needs, responsivity factors, and structured intervention fit together conceptually. While not itself a primary empirical study, it is one of the most authoritative sources for explaining the framework accurately. How to use in Section 5: Use for definitions and conceptual framing, but pair it with the empirical review sources in this section.

3. Bouchard, J., Wong, J. S., & Gendron, A. (2024). An updated evidence synthesis on the Risk-Need-Responsivity (RNR) model: An umbrella review. *Journal of Criminal Justice*, 92, Article 102199. Year/journal/article number should be verified.

Plain-English summary: This newer umbrella review reportedly synthesizes the quality and consistency of the evidence base behind RNR principles. It is useful because it updates the older validation literature and shows that RNR remains the dominant evidence-supported model in correctional rehabilitation. It can also help you acknowledge where evidence is stronger versus thinner. How to use in Section 5: Use as your “current state of the evidence” citation for RNR’s empirical support.

4. Ward, T., Melser, J., & Yates, P. M. (2007). Reconstructing the Risk-Need-Responsivity model: A theoretical elaboration and evaluation. *Aggression and Violent Behavior*, 12(2), 208-228. <https://doi.org/10.1016/j.avb.2006.07.001>

Plain-English summary: This article critiques and extends RNR by arguing that good correctional rehabilitation should address not only risk reduction but also the person’s goals, agency, and capacity to

build a meaningful prosocial life. It is important because it captures the intellectual space from which the Good Lives Model (GLM) developed. It does not reject RNR outright; instead, it highlights what can be added to make treatment more engaging and humane. How to use in Section 5: Use this to introduce why a strengths-based or integrative framework became attractive alongside RNR.

5. Ward, T., & Maruna, S. (2007). *Rehabilitation: Beyond the risk paradigm*. Routledge. Book source; useful for GLM/desistance framing.

Plain-English summary: This book is central to the GLM and strengths-based rehabilitation literature. It argues that people desist more successfully when interventions help them pursue prosocial ways of meeting core human needs rather than only suppressing risk. It is especially relevant to your dissertation because desistance-oriented work with emerging adults often requires more than risk management alone. How to use in Section 5: Use to support a strengths-based, developmentally informed rationale for integrated treatment.

3) Evidence-based interventions in correctional settings (CBT, DBT, schema therapy, MI, and related approaches)

1. Landenberger, N. A., & Lipsey, M. W. (2005). The positive effects of cognitive-behavioral programs for offenders: A meta-analysis of factors associated with effective treatment. *Journal of Experimental Criminology*, 1(4), 451-476. <https://doi.org/10.1007/s11292-005-3541-7>

Plain-English summary: This meta-analysis found that CBT-oriented programs produce meaningful reductions in recidivism across offender populations. The paper also identifies conditions under which CBT works better, such as higher quality implementation and stronger adherence to evidence-based design features. It is one of the best sources for establishing CBT as a correctional gold-standard intervention. How to use in Section 5: Use this as your primary support for CBT as a core intervention in correctional treatment.

2. Beaudry, G., Yu, R., Långström, N., & Fazel, S. (2021). Effectiveness of psychological interventions in prison to reduce recidivism: A systematic review and meta-analysis of randomized controlled trials. *The Lancet Psychiatry*, 8(9), 759-773. [https://doi.org/10.1016/S2215-0366\(21\)00170-X](https://doi.org/10.1016/S2215-0366(21)00170-X)

Plain-English summary: This review of randomized controlled trials found that psychological interventions delivered in prison can reduce recidivism after release, with stronger effects in some intervention types than others. The paper is especially valuable because it uses a more rigorous study-design threshold than many older correctional reviews. It gives you a modern, high-impact evidence base for structured prison treatment. How to use in Section 5: Use this to support the broader claim that manualized psychological interventions can work in custodial settings.

3. Bernstein, D. P., Keulen-de Vos, M., Clercx, M., de Vogel, V., Kersten, G. C., Lancel, M., Jonkers, P. P., van den Broek, E., Bernie, C., & Arntz, A. (2021). Schema therapy for violent personality-disordered offenders: A randomized clinical trial. *Psychological Medicine*, 51(6), 1001-1013. Page range should be verified.

Plain-English summary: This randomized trial tested schema therapy with violent offenders who had personality disorders and found improvements that support the idea that entrenched maladaptive patterns are treatable, even in high-risk forensic populations. It is one of the strongest direct sources for schema therapy in a forensic context. The study is especially relevant if you want to argue for going beyond surface-level cognitive restructuring to deeper developmental schemas and modes. How to use in Section 5: Use this as your flagship forensic schema therapy citation.

4. Gonçalves, R. A., et al. (2025). Effectiveness of motivational interviewing with justice-involved people: A systematic review and meta-analysis. Psychosocial Intervention / Intervención Psicosocial. Exact APA details should be verified.

Plain-English summary: This recent systematic review/meta-analysis suggests that motivational interviewing can improve engagement and support behavior change work with justice-involved clients, especially where ambivalence, distrust, or low readiness are major barriers. That matters in correctional settings because treatment is often mandated and participants are not automatically invested. MI is therefore less a standalone cure and more a crucial engagement tool within broader intervention packages. How to use in Section 5: Use this to justify motivational work as a responsivity and engagement strategy inside an integrated model.

5. Shelton, D., Sampl, S., Kesten, K., Zhang, W., & Trestman, R. L. (2009). Treatment of impulsive aggression in correctional settings using dialectical behavior therapy. Criminal Justice and Behavior, 36(2), 175-191. APA details should be verified.

Plain-English summary: This study supports DBT-informed intervention for incarcerated individuals with high emotion dysregulation, impulsivity, and aggression. DBT is particularly relevant in populations marked by trauma exposure, self-harm, conflict, and poor distress tolerance. It can complement CBT by targeting emotional regulation and crisis behavior more directly. How to use in Section 5: Use this to show why DBT skills may be useful when standard CBT alone is not enough for dysregulated clients.

6. Kigerl, A., & Hamilton, Z. (2024). Effectiveness of dialectical behavior therapy in a forensic population: A meta-analysis. Criminal Justice and Behavior. Exact APA details should be verified.

Plain-English summary: This newer meta-analysis indicates growing empirical support for DBT in forensic and correctional populations, especially for emotional and behavioral dysregulation outcomes. It is helpful because DBT evidence in corrections has historically been scattered and under-synthesized. This source lets you discuss DBT as promising and increasingly evidence-based, while still noting the literature is newer than the CBT literature. How to use in Section 5: Use this as a current synthesis when describing DBT as a relevant adjunctive intervention.

4) Effectiveness of correctional interventions (meta-analyses and other empirical studies)

1. Lipsey, M. W., & Cullen, F. T. (2007). The effectiveness of correctional rehabilitation: A review of systematic reviews. Annual Review of Law and Social Science, 3, 297-320. <https://doi.org/10.1146/annurev.lawsocsci.3.081806.112833>

Plain-English summary: Across many meta-analyses, the overall pattern is that treatment can reduce reoffending, and programs aligned with effective intervention principles perform best. This source is especially useful because it compares sanctions/surveillance approaches with rehabilitation approaches across a broad literature. It gives you the “big picture” effect story. How to use in Section 5: Use as an umbrella citation for the overall effectiveness of rehabilitation programming.

2. Landenberger, N. A., & Lipsey, M. W. (2005). The positive effects of cognitive-behavioral programs for offenders: A meta-analysis of factors associated with effective treatment. *Journal of Experimental Criminology*, 1(4), 451-476. <https://doi.org/10.1007/s11292-005-3541-7>

Plain-English summary: This paper shows that one of the strongest evidence bases in corrections is for CBT-based programming. It also demonstrates that effectiveness is not just about the label “CBT,” but about implementation quality, treatment intensity, and fit. That makes it useful both as an intervention-effectiveness source and as a bridge into program design issues. How to use in Section 5: Use to support the claim that CBT is not just popular—it is empirically one of the most defensible correctional interventions.

3. Beaudry, G., Yu, R., Långström, N., & Fazel, S. (2021). Effectiveness of psychological interventions in prison to reduce recidivism: A systematic review and meta-analysis of randomized controlled trials. *The Lancet Psychiatry*, 8(9), 759-773. [https://doi.org/10.1016/S2215-0366\(21\)00170-X](https://doi.org/10.1016/S2215-0366(21)00170-X)

Plain-English summary: This RCT-based meta-analysis provides relatively conservative evidence that prison-based psychological interventions can reduce recidivism. Because the authors restricted the review to randomized trials, the findings carry extra weight when addressing skepticism about treatment effectiveness. It also helps you avoid overstating effects, which strengthens credibility. How to use in Section 5: Use this as a rigorous modern source to counter “rehabilitation doesn’t work” claims.

4. Lösel, F., & Schmucker, M. (2005). The effectiveness of treatment for sexual offenders: A comprehensive meta-analysis. *Journal of Experimental Criminology*, 1(1), 117-146. <https://doi.org/10.1007/s11292-004-6466-7>

Plain-English summary: Although focused on sexual offending, this meta-analysis is often cited as evidence that even populations once viewed as “untreatable” show better outcomes with structured intervention than without it. It strengthens the broader rehabilitation argument by showing effects in a high-stigma, high-risk subgroup. It also illustrates how treatment effectiveness varies by modality and implementation quality. How to use in Section 5: Use as a “hard case” example showing that treatment can work even with highly challenging forensic populations.

5) Programmatic elements that predict success (dosage, alliance, aftercare, staff training, fidelity, responsivity)

1. Bourgon, G., Bonta, J., Rugge, T., Scott, T.-L., Yessine, A. K., Gutierrez, L., & Li, J. (2010). The role of program design, implementation, and evaluation in evidence-based “real world” community supervision. *Federal Probation*, 74(1), 2-15. Nontraditional outlet; useful practice-research bridge.

Plain-English summary: This paper emphasizes that effective intervention depends on more than choosing the right theory; delivery quality, staff behavior, fidelity, and evaluative feedback matter enormously. It is useful because it reflects the reality that evidence-based correctional practice often fails at the implementation stage rather than the conceptual stage. The article pairs well with RNR and core correctional practices literature. How to use in Section 5: Use to argue that implementation quality is a treatment variable, not a side issue.

2. Bonta, J., Bourgon, G., Rugge, T., Scott, T.-L., Yessine, A. K., Gutierrez, L., & Li, J. (2011). An experimental demonstration of training probation officers in evidence-based community supervision. *Criminal Justice and Behavior*, 38(11), 1127-1148. APA details should be verified.

Plain-English summary: This study showed that when probation officers were trained in evidence-based supervision practices, client outcomes improved relative to usual supervision. It is one of the best empirical examples that staff skills and supervisory style directly affect outcomes. In other words, evidence-based practice is not just about the curriculum; it is also about the people delivering it. How to use in Section 5: Use this when discussing staff training, coaching, and interaction style as mechanisms of program success.

3. Lowenkamp, C. T., Latessa, E. J., & Holsinger, A. M. (2006). The risk principle in action: What have we learned from 13,676 offenders and 97 correctional programs? *Crime & Delinquency*, 52(1), 77-93. <https://doi.org/10.1177/0011128705281747>

Plain-English summary: This study found that program effects depend heavily on matching treatment intensity to risk level. Higher-risk individuals benefited more from intensive services, while low-risk individuals did not benefit in the same way and could even do worse. It is a core source for service dosage and risk matching. How to use in Section 5: Use this to support arguments about dosage and intensity needing to be calibrated rather than simply increased.

4. Dowden, C., & Andrews, D. A. (2004). The importance of staff practice in delivering effective correctional treatment: A meta-analytic review of core correctional practice. *International Journal of Offender Therapy and Comparative Criminology*, 48(2), 203-214. <https://doi.org/10.1177/0306624X03257765>

Plain-English summary: This meta-analysis found that staff who use effective interpersonal and cognitive-behavioral practices produce better outcomes. It supports the view that therapeutic alliance, reinforcement, modeling, problem solving, and respectful confrontation are not “soft” extras—they are part of what makes correctional treatment work. This is a very useful bridge between responsivity and alliance. How to use in Section 5: Use this to support your discussion of alliance, staff quality, and relational responsivity.

5. James, C., Stams, G. J. J. M., Asscher, J. J., De Roo, A. K., & van der Laan, P. H. (2013). Aftercare programs for reducing recidivism among juvenile and young adult offenders: A meta-analytic review. *Clinical Psychology Review*, 33(2), 263-274. <https://doi.org/10.1016/j.cpr.2012.10.013>

Plain-English summary: This meta-analysis found that aftercare programs can improve outcomes for juvenile and young adult offenders after release, though effects vary by program type and quality. It matters because it shows treatment gains can dissipate without continuity into the community. For an emerging-adult population, this is especially important because instability after release is common. How to

use in Section 5: Use this to justify aftercare and continuity-of-care components in a residential manual.

6. de Jonge, F., & colleagues (2024). Working alliance, risk, and recidivism: A community-based corrections study. Criminal Justice and Behavior. Exact APA details should be verified.

Plain-English summary: This recent study suggests that client-rated working alliance is meaningfully related to outcomes in community corrections. Even in mandated settings, how the client experiences the helping relationship appears to matter. This supports the argument that alliance is compatible with accountability and can function as a responsivity mechanism. How to use in Section 5: Use this as a current citation for therapeutic alliance in justice settings.

6) Challenges and limitations in correctional programming (institutionalization, lack of developmental programming, resistance to treatment, lack of individualized/developmentally tailored content for emerging adults)

1. Haney, C. (2003). The psychological impact of incarceration: Implications for post-prison adjustment. In J. Travis & M. Waul (Eds.), Prisoners once removed (pp. 33-66). Urban Institute Press. Book chapter; not peer-reviewed journal, but widely cited and highly useful.

Plain-English summary: Haney details how incarceration can foster dependency, emotional constriction, social withdrawal, and other institutional adaptations that later interfere with functioning outside prison. This is a strong source for describing “institutionalization” or prisonization as a barrier to rehabilitation. It helps explain why treatment has to address not only crime-related cognition but also adaptation to coercive environments. How to use in Section 5: Use this to explain why correctional context itself can undermine treatment responsiveness and postrelease success.

2. Morgan, R. D., Kroner, D. G., Mills, J. F., Serna, C., & McDonald, B. (2013). Treating offenders with mental illness: A research synthesis. Law and Human Behavior, 37(1), 37-50. APA details should be verified.

Plain-English summary: This synthesis reviews the difficulty of implementing effective treatment with justice-involved individuals who present with serious mental health needs, co-occurring disorders, and multiple responsivity barriers. It is helpful for showing why “one size fits all” correctional programs often fail. The article also supports the case for individualized and clinically informed intervention planning. How to use in Section 5: Use this when discussing responsivity barriers, heterogeneity, and limitations of generic programming.

3. Schubert, C. A., Mulvey, E. P., & Pitzer, L. (2016). Differentiating serious adolescent and emerging adult offenders. Justice Quarterly, 33(4), 1-27. APA details should be verified.

Plain-English summary: This work highlights developmental heterogeneity among late adolescents and emerging adults involved in the justice system. It supports the idea that this age band cannot be treated as simply “older juveniles” or “fully mature adults.” That makes it especially relevant to your dissertation’s focus

on emerging adulthood. How to use in Section 5: Use this to justify developmentally tailored content rather than standard adult programming.

4. Child Trends. (2024). Supporting emerging adults developmental framework. Report, not peer-reviewed; use only as supplementary context.

Plain-English summary: This framework summarizes why emerging adults in the legal system show high recidivism, high service needs, and major developmental mismatches with standard adult systems. It is not a primary peer-reviewed source, but it gives a concise and current summary of the service gap. It can help you frame why a manual designed specifically for emerging adults fills a real systems problem. How to use in Section 5: Use only as a supplemental policy-context citation, not as one of your core empirical anchors.

5. Polaschek, D. L. L., & Ross, E. C. (2010). Do early therapeutic alliance, motivation, and stages of change predict therapy change for high-risk, psychopathic violent prisoners? Criminal Behaviour and Mental Health, 20(2), 100-111. APA details should be verified.

Plain-English summary: This study is useful for demonstrating that treatment resistance, motivation, and alliance meaningfully affect outcomes even among very difficult forensic populations. It pushes back against the idea that non-engagement is just “defiance” and instead shows it is a clinically important treatment variable. That makes it very relevant to responsivity and treatment planning. How to use in Section 5: Use this when discussing resistance to treatment and the need for engagement-focused intervention.

7) Rationale for integrating CBT and schema therapy

1. Bernstein, D. P., Keulen-de Vos, M., Clercx, M., de Vogel, V., Kersten, G. C., Lancel, M., Jonkers, P. P., van den Broek, E., Bernie, C., & Arntz, A. (2021). Schema therapy for violent personality-disordered offenders: A randomized clinical trial. Psychological Medicine, 51(6), 1001-1013. Page range should be verified.

Plain-English summary: This trial supports the idea that deeper personality-linked schemas and modes can be changed in forensic populations, not just surface-level thoughts or behaviors. For your purposes, it suggests that schema-focused work may be valuable when clients have chronic, rigid, trauma-linked patterns that standard CBT does not fully address. This is one of the best direct forensic sources for making the integration case. How to use in Section 5: Use this to argue that schema work may enhance treatment for clients with entrenched maladaptive patterns.

2. Masley, S. A., Gillanders, D. T., Simpson, S. G., & Taylor, M. A. J. (2012). A systematic review of the evidence base for schema therapy. Cognitive Behaviour Therapy, 41(3), 185-202. <https://doi.org/10.1080/16506073.2011.614274>

Plain-English summary: This review concludes that schema therapy has a growing evidence base, particularly for chronic and personality-related problems that often complicate treatment in justice populations. It is useful for explaining why schema therapy can add something beyond standard CBT when problems are longstanding, relational, and emotionally rooted. While not correctional-specific, it supports the clinical logic of integration. How to use in Section 5: Use this to explain why schema therapy may deepen or extend CBT for more complex cases.

3. Bamelis, L. L. M., Evers, S. M. A. A., Spinhoven, P., & Arntz, A. (2014). Results of a multicenter randomized controlled trial of the clinical effectiveness of schema therapy for personality disorders. *American Journal of Psychiatry*, 171(3), 305-322. <https://doi.org/10.1176/appi.ajp.2013.12040518>

Plain-English summary: This RCT showed schema therapy can be effective for personality disorders, a set of problems that are common in forensic and correctional samples. The relevance here is indirect but strong: if schema therapy helps with chronic maladaptive interpersonal and self-regulation patterns, it may address treatment barriers that weaken pure skills-based approaches. This makes it a good source for the “why integrate?” argument. How to use in Section 5: Use this to support the idea that schema-informed work may be useful for clients with personality pathology and rigid developmental patterns.

4. Rafaeli, E., Bernstein, D. P., & Young, J. (2011). *Schema therapy: Distinctive features*. Routledge. Book source; use for conceptual explanation rather than empirical proof.

Plain-English summary: This source helps explain the mechanisms schema therapy targets: early maladaptive schemas, coping styles, and modes. That framework is useful when articulating why CBT plus schema therapy might be more clinically complete for clients with trauma, personality dysfunction, and entrenched criminal thinking. Use it for theory, not as a primary outcomes source. How to use in Section 5: Use sparingly for conceptual language when defining what schema therapy adds to CBT.

8) Trauma-informed care in justice/correctional populations

1. Branson, C. E., Baetz, C. L., Horwitz, S. M., & Hoagwood, K. E. (2017). Trauma-informed juvenile justice systems: A systematic review of definitions and core components. *Psychological Trauma: Theory, Research, Practice, and Policy*, 9(6), 635-646. <https://doi.org/10.1037/tra0000330>

Plain-English summary: This systematic review clarifies what trauma-informed care means in justice settings and identifies core components needed at the system level. It is especially useful because trauma-informed care is often discussed vaguely; this article gives you a more precise and research-grounded definition. Even though it focuses on juvenile justice, the principles generalize well to emerging-adult and correctional settings. How to use in Section 5: Use this to define trauma-informed care and identify the operational components you may want in a residential manual.

2. Baranyi, G., Cassidy, M., Fazel, S., Priebe, S., & Mundt, A. P. (2022). Trauma exposure and mental health of prisoners and ex-prisoners: A systematic review and meta-analysis. *Clinical Psychology Review*, 91, 102087. <https://doi.org/10.1016/j.cpr.2021.102087>

Plain-English summary: This meta-analysis shows that prisoners and ex-prisoners have very high levels of trauma exposure and strong associations between trauma and later mental health problems. It gives you a strong empirical basis for arguing that trauma is not peripheral but central in correctional populations. That matters because untreated trauma can undermine engagement, emotion regulation, and behavior change. How to use in Section 5: Use this to justify trauma screening and trauma-informed intervention design as clinically necessary, not optional.

3. Kerig, P. K., & Becker, S. P. (2015). Trauma and girls' delinquency. In APA handbook / justice-involved youth literature. Exact source details should be verified if used.

Plain-English summary: The trauma-delinquency literature shows how victimization, dysregulation, and survival-based coping can shape justice involvement, particularly among girls and young women. This line of work is useful if your section needs to explain why behavior in justice settings may reflect trauma adaptation rather than simple oppositionality. While some sources are youth-focused, the conceptual implications extend to emerging adults. How to use in Section 5: Use to support gender-responsive and trauma-informed tailoring, especially for female or mixed-gender populations.

4. Miller, N. A., Najavits, L. M. (2012). Creating trauma-informed correctional care: A balance of goals and environment. European Journal of Psychotraumatology, 3, 17246. <https://doi.org/10.3402/ejpt.v3i0.17246>

Plain-English summary: This article explains how trauma-informed correctional practice must account for both clinical needs and custodial realities. It highlights environmental triggers, coercion, shame, and safety as issues that can either support or undermine treatment. It is particularly useful for translating trauma concepts into concrete correctional practice. How to use in Section 5: Use this when discussing how a residential model can be trauma-informed without losing structure or accountability.

9) Cultural and contextual responsivity in intervention design

1. Wormith, J. S., Althouse, R., Simpson, M., Reitzel, L. R., Fagan, T. J., & Morgan, R. D. (2007). The rehabilitation and reintegration of offenders: The current landscape and some future directions for correctional psychology. Criminal Justice and Behavior, 34(7), 879-892. APA details should be verified.

Plain-English summary: This article discusses the need to translate broad evidence-based principles into interventions that actually fit clients' social, cultural, and contextual realities. It is useful for arguing that responsivity is more than learning style; it includes relational, environmental, and identity-related fit. That broader interpretation supports culturally and contextually responsive design. How to use in Section 5: Use this to broaden the discussion of responsivity beyond generic CBT delivery.

2. Jones, N. J., Brown, S. L., Wanamaker, K. A., & Greiner, L. E. (2014). A quantitative exploration of responsivity factors and offender change. Psychology, Crime & Law, 20(9), 873-893. APA details should be verified.

Plain-English summary: This study examines how responsivity factors shape treatment change, reinforcing that intervention outcomes depend partly on barriers outside the classic criminogenic-need list. These may include motivation, mental health, interpersonal style, and contextual fit. It is useful when arguing for individualized and culturally/contextually responsive intervention planning. How to use in Section 5: Use this to support a nuanced discussion of specific responsivity factors.

3. Shepherd, S. M., Delgado, R. H., Sherwood, J., & Paradies, Y. (2018/2021). Considering practice frameworks for culturally diverse populations in correctional settings. Aggression and Violent Behavior. Exact year/details should be verified.

Plain-English summary: This work addresses how mainstream correctional models often insufficiently account for cultural difference, especially for Indigenous and other marginalized populations. It argues that responsivity should include collaboration, cultural knowledge, and adaptation of intervention frameworks rather than assuming universal fit. This is important if you want to frame integrated treatment as responsive to real-world diversity. How to use in Section 5: Use this as a direct support for culturally responsive correctional intervention design.

4. Viglione, J., Blasko, B. L., & Taxman, F. S. (2019). A systems perspective on responsivity in correctional treatment and supervision. Federal Probation / related justice outlet. Exact APA details should be verified.

Plain-English summary: This line of work emphasizes that responsivity is shaped not only by the client but also by the system, staff, environment, and available resources. That is useful because it lets you talk about contextual responsivity as an organizational issue rather than just an individual deficit. It fits well with an integrated residential model where milieu and staff culture matter. How to use in Section 5: Use this to support the argument that intervention design must match both person and setting.

Suggested “best core set” if you want to draft fast

If Destyne only wants the highest-yield sources to start writing immediately, these are the ones I'd pull first:

- 1 Lipsey & Cullen (2007) – broad rehabilitation effectiveness and historical shift away from punishment-only models.
- 2 Andrews, Bonta, & Hoge (1990) – foundational RNR framework.
- 3 Landenberger & Lipsey (2005) – CBT effectiveness in offender treatment.
- 4 Beaudry et al. (2021) – rigorous RCT-based evidence that prison psychological interventions can reduce recidivism.
- 5 Bernstein et al. (2021) – direct forensic schema therapy RCT.
- 6 Dowden & Andrews (2004) – staff practice/alliance/core correctional practice.
- 7 Lowenkamp, Latessa, & Holsinger (2006) – dosage/risk matching.
- 8 James et al. (2013) – aftercare for juvenile/young adult offenders.
- 9 Branson et al. (2017) and Baranyi et al. (2022) – trauma-informed and trauma-prevalence justification.
- 10 Shepherd et al. – cultural responsivity.

Writing note for Section 5

A clean way to structure the argument is:

- 1 Punishment alone has weak evidence ->
- 2 Rehabilitation works best when structured by evidence-based principles (RNR) ->
- 3 CBT is the best-established intervention base ->

- 4 But many justice-involved emerging adults also present with trauma, personality-linked maladaptive schemas, and emotion dysregulation ->
- 5 Therefore, an integrated model that keeps CBT's structure while incorporating schema-focused and trauma-informed elements may be more responsive, developmentally appropriate, and clinically complete.

That framing fits your dissertation topic really damn well because it ties desistance, emerging adulthood, responsivity, and treatment integration into one coherent rationale.